



February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
U.S. Department of Health & Human Services
330 C St., SW, Floor 7
Washington, DC 20201

Submitted electronically at www.regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Assistant Secretary Keane:

On behalf of our 100,000 member physical therapists, physical therapist assistants, and students of physical therapy, the American Physical Therapy Association appreciates the opportunity to submit comments in response to the Department of Health and Human Services' [Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care](#). APTA is dedicated to building a community that advances the physical therapy profession to improve the health of society. As experts in rehabilitation, pre-habilitation, and habilitation, physical therapists play a unique role in society in prevention, wellness, fitness, health promotion, and management of disease and disability for individuals across the age span, helping individuals improve overall health and prevent the need for avoidable health care services. Physical therapists' roles include education, direct intervention, research, advocacy, and collaborative consultation. These roles are essential to the profession's vision of transforming society by optimizing movement to improve the human experience.

Artificial Intelligence Can Augment Physical Therapist Practice

APTA is pleased to see that HHS alongside the Assistant Secretary for Technology Policy and Office of the National Coordinator for Health Information Technology is seeking stakeholder feedback on how to appropriately regulate, reimburse, and encourage research and development of artificial intelligence in clinical care. APTA believes digital health technologies, digital therapeutics, and artificial intelligence have the potential to augment physical therapist practice by expanding access, enhancing care delivery models, promoting safety, reducing administrative burden, and improving outcomes. As part of this belief, [we continue to support the ethical development and integration of AI into clinical care when it reduces administrative burden and enhances physical therapist practice, education, and research for the best interest of patients and the profession](#). But to ensure that AI is used in the best interests of the patient, we emphasize the following principles:

- Physical therapists, in collaboration with the patient or client, shall determine the optimal modes to access physical therapist services, whether this is in person, remotely, or some combination.
- A physical therapist is responsible for all aspects of patient and client management.
- Physical therapist services shall be conducted in a manner that allows for patient and client engagement and supports the therapeutic alliance.
- The physical therapist and the physical therapist assistant adhere to scope of practice, including requirements that are based on the jurisdiction in which the patient or client is located when receiving physical therapist services.
- Physical therapist services shall be provided consistent with appropriate direction and supervision requirements of assistive personnel and other support personnel, as well as all relevant Association policies, positions, and binding ethical documents.
- Physical therapy is represented, provided, and promoted only when it meets term and title protections as defined in state law and in accordance with federal health, communications, and trade authorities.

Acknowledging these principles, we strongly assert that technology that utilizes artificial intelligence should only be used as an extension of the physical therapist practice, not as a replacement of the physical therapist. With this in mind, we provide our responses to the questions posed in the RFI.

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Limited Regulatory Guidance: The current AI legal and regulatory landscape is a patchwork of guidance that places the onus on industry to evaluate and ensure compliance. Additionally, there is little to no assistance given to health care providers on how to deploy AI safely and effectively in clinical settings. It would be inappropriate to say that federal agencies have not exercised any regulatory authority over AI, particularly as it relates to safety and security, however [most administrative actions to date have focused on enforcement of existing regulatory authorities and voluntary commitments from the industry](#) as opposed to creating new laws and regulations. While all positive steps towards alignment, task forces, action plans and frameworks set standards for best practices that are not enforceable by law. This lack of oversight at the federal level concerning the development, prohibition, or restriction of AI has [forced states to enact their own laws](#) which only further complicates the policy environment.

The reason for this lack of oversight is because no laws to date have provided regulatory authorities to a specific department or agency for the development of AI, use of AI, or prohibitions on AI. While the Food and Drug Administration has regulatory authority over medical devices, AI technology is only considered “software as a medical device” if it makes claims about diagnosing, treating, preventing, or curing a disease or medical condition, provides recommendations or actions that significantly impact clinical decision-making, processes patient-specific medical data and provides outputs that are time-critical or could lead to a clinical intervention and does not allow or clearly enable the clinician to independently review how the decision is made. Consequently, AI devices used in clinical care are not regulated if they help with administration, record keeping, scheduling, reminders, or similar nonclinical workflows, and most notably, if it is a clinical decision support tool that does not use medical images,

make directives, or work with information that is normally communicated between providers, and if the basis for any recommendations the device provides can be independently reviewed by a clinician. This criteria leaves a glaring gap between what is current regulated and what should be regulated, namely by [excluding many clinical decision support software tools from the definition of medical device](#). Thus, while holding status as the agency that regulates medical devices, in actuality, the FDA is only regulating a small portion of AI devices that are used in clinical care. This lack of clear federal oversight creates fear and discourages providers from using AI-powered devices.

Inability to Identify High-Quality AI Tools: Because there is limited federal oversight of AI use in healthcare settings, clinicians lack clear mechanisms by which to identify high-quality technology. To fill this gap, APTA partnered with [ORCHA, a digital health evaluation and distribution platform](#), to encourage appropriate use of digital health tools. With ORCHA's assistance, APTA launched a [Digital Health App Library](#) containing digital health tools that have been independently reviewed for safety, effectiveness, and quality. ORCHA evaluates the tools based on a [digital health assessment framework](#) that focuses on three key evaluation criteria across 400 standards: data and privacy, clinical assurance and safety, and usability and accessibility. An additional criterion—technical security and stability—has been newly added and is not yet visible on all applications. Notably, ORCHA reassesses digital health apps every time they are updated.

While APTA has used ORCHA to meet a clear need, the system is far from perfect. ORCHA's digital health advisory committee is currently developing an AI evaluation tool that is set to launch in the coming months but until then, the evaluation process does not currently assess AI use and cannot be relied upon to adequately evaluate the AI mechanisms within a digital platform. Additionally, many app developers have not made it clear who bears liability if there is an adverse event which may increase provider hesitancy. Until there is a better mechanism that allows providers to distinguish between low- and high-quality AI technologies, many may choose not to use them at all. To avoid this, APTA encourages HHS to use an interagency approach to establish a public registry of AI tools used in clinical care, including information on their intended use, validation evidence, and performance metrics. This would function similarly to FDA's medical device databases but for non-medical AI devices where there is an existing gap.

Liability Remains Unclear: Historically, physical therapists have been fully responsible for all aspects of patient management, but PTs cannot continue to bear the brunt of all professional liability if AI plays an increasing role in the delivery and oversight of care. The therapist knows that it is their license that can be taken away in instances of negligence, injury, or improper treatment, so if HHS wants to increase the use of AI in clinical care, there needs to be clear distinction between the therapist's and technology's liability. Unless these clear delineations in legal and ethical responsibilities are made, providers will continue to express hesitancy around fully adopting AI technology in their clinics.

Lack of Infrastructure: Many providers have also yet to adopt AI technologies because they cannot afford the underlying infrastructure required for AI adoption, including investments in interoperability,

workforce training, and technical assistance. Physical therapists, in particular, have faced significant challenges related to interoperability, namely the acquisition of certified health record technology. This limited adoption of standardized data exchange formats hinders the flow of critical patient information across care transitions and limits the utility of AI devices. If clinicians are not supported in the uptake of AI, the digital divide will continue to create inequitable access to AI use in clinical care.

Reimbursement Barriers: Fee-for-service payment structures provide limited incentives for providers to invest in AI tools, even when such tools could improve outcomes and reduce costs over time. Coverage and reimbursement decisions for AI-enabled services are slow, and current payment models do not adequately account for the value of AI interventions. To solve this, alternative payment models should incentivize the use of high-value AI interventions that improve patient outcomes. APTA was pleased to see the introduction of the [Advancing Chronic Care with Effective, Scalable Solutions Model](#) which offers payments to providers who use technology to treat a range of chronic conditions, and we are eager to see whether the model's deployment will increase the uptake of AI technology.

Limited Understanding of AI Risks, Capabilities and Limitations: At this time, the safe and proper use of digital tools, including AI, is not a mandated part of a physical therapist's training and curriculum. As a result, the extent to which certain clinicians are familiar with AI risks and benefits or data safety principles varies immensely. And while the use of technology, whether it utilizes AI or not, is a risk to all clinicians who are unfamiliar with it, the risk is higher for newer clinicians who may become deskilled by the overuse of and overreliance on AI. This could lead to issues with long-term workforce competency due to skill atrophy, passive learning, and an overreliance on false expertise rather than clinical training. Because the value of AI outputs is highly dependent on the questions being asked, clinician users must understand how to properly engage with the model to receive the best output. If utilized by clinicians who are untrained in AI systems, there could be catastrophic repercussions. To avoid this, APTA encourages HHS to support professional societies in developing AI competency standards and continuing education requirements that help develop AI literacy.

For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Robust Data Training in Large Language Models: AI applications utilize large language models that rely on learning algorithms that are trained on massive data sets. Medical LLMs [require vast and diverse datasets encompassing clinical records, medical literature, healthcare guidelines, and domain-specific knowledge resources](#). The quality, diversity, and representativeness of the training data directly impacts the model's performance and whether the model's outputs will align with current clinical practice. As a result, considerable focus must be placed on ensuring that LLMs are trained on robust, high-quality research and existing patient data, and that this data is continuously updated as the science.

Furthermore, reinforcement learning from human feedback is also necessary to align model outputs with current guidelines, enhancing the relevance and safety of generated content. Without adequate training with robust data and the ability to learn from human feedback, the use of AI in clinical care could exacerbate existing bias and discrimination.

A landmark [2019 study](#) found that a commercially available algorithm used to predict health care needs for more than 100 million people was biased against Black patients. The algorithm relied on past health care spending to predict future health needs but with less access to care, Black patients often spent less. As a result, Black patients had to be much sicker to be recommended for extra care under the algorithm. If adequate quality standards are not placed on AI data acquisition and training before the model is deployed, the device will inadvertently seek out patterns which could lead to biased outcomes. There must be systems in place to both ensure that only unbiased, high-quality data is used in AI models and to recognize when inappropriate recommendations are made.

To avoid any risk of bias and ensure LLMs have clinical integrity, APTA urges HHS to partner with ASTP/ONC to create pre-deployment evaluation methods including but not limited to prospective validation, bias auditing, and workflow integration testing. And even after a model has been trained, its performance must be continuously monitored and patient outcomes should be surveilled to ensure that the model is functioning as intended. In this, HHS should also require developers to perform routine dataset auditing that looks at performance across different patient demographics to prevent patients from receiving biased or clinically unproven outputs.

Output Transparency: High-performance AI models can operate as “black boxes,” making their outputs hard to interpret. This limits the provider’s ability to review clinical evidence that supports any treatment recommendations the device makes. To ensure that clinicians trust AI as their clinical assistant, all AI tools used in clinical care should be evaluated for their ability to provide clinically meaningful explanations of treatment recommendations.

The most promising way to inform the provider—who is the end user—and future developers of how a given AI tool was built, and the algorithms underlying its design, is through model cards. They function differently than a typical user manual because AI is a living breathing tool that can, and should, be revised over time. By spelling out how the algorithms underlying the technology were developed and their intended use, model cards help clinicians understand the capabilities and limits of the models which can help them best use the platform and educate patients accordingly. To ensure this level of transparency, APTA recommends that HHS require the use of model cards as a transparency mechanism within all AI tools used to provide care under federally regulated insurance programs.

Privacy, Security, and HIPAA Compliance: The use of AI in clinical care raises new privacy and security concerns that have not yet been addressed through rulemaking and go beyond that of traditional care. Such concerns include: whether AI-generated clinical insights constitute protected health information under HIPAA, how consent should be obtained for using patient data, security standards for model

storage, transmission, and updates, and privacy risks when AI tools use health data from non-HIPAA covered wearables and apps. While there are many ways to ensure these concerns are addressed, including but not limited to Business/ Associate Agreements, HIPAA eligible hosting, encryption, and zero data-retention controls, it is currently unclear which compliance standards non-medical AI devices are subject to. To decrease ambiguity, HHS should clearly state which existing privacy and security laws and regulations AI must comply with. Specifically, HHS should issue guidance clarifying HIPAA applicability to AI systems and establish security standards for all AI tools used in clinical care.

Liability Concerns: When an AI tool recommends a clinical intervention that results in patient harm, liability questions arise: Is the provider liable for following the recommendation? Is the AI vendor liable for the algorithm's output? Is the health system liable for deploying an inadequately validated tool? Current medical malpractice frameworks do not clearly address AI-mediated decision-making. To avoid this confusion, some companies, such as Doctronic, have gone as far as to [create malpractice insurance for the AI system so that it may hold the same level of accountability as a physician](#). Is this to be expected of all AI tools, autonomous or not? These questions must be answered to encourage AI innovation and use in clinical care. APTA asks HHS to work with state medical boards, professional liability insurers, and legal experts to develop guidance on shared accountability models.

Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

If properly regulated and utilized appropriately, high-quality AI technology can be paired with the human judgement of a clinician to enable better care, tackle numerous challenges that exist within the larger healthcare system, and reduce strain on the clinician workforce. In physical therapist practice, AI tools are already being deployed to reduce provider paperwork and health data complexity and fragmentation to optimize patient outcomes.

The following tools have met or exceed performance and have the greatest potential to improve health care outcomes:

Clinical Documentation and Administrative Support: The healthcare system utilizes enormous amounts of textual data in the form of medical records, research literature, clinical guidelines, and patient communications. The vast quantity of this data and the need for it to be reviewed in a timely manner represents a terrific opportunity for the use of AI, or more specifically, LLMs. Rather than utilize the time of a skilled clinician, administrative tasks, such as scheduling, billing, documentation drafting, and literature review, could be handled by AI. Using AI to automate these processes could save provider time, reduce burden, and pose little to no risk because they have no ability to alter clinical decisions as long as the provider is able to review any final outputs. And if clinicians are freed up to practice at the top of their licensure, they can see more patients or handle more complex cases.

A fitting example is AI-powered documentation that produces written summaries of patient encounters. APTA recently published a [practice advisory on AI-ambient tools](#) which operate discreetly in the background and use artificial intelligence to automatically capture, transcribe, and summarize patient-provider interactions into structured clinical notes. Administrative optimization is another means by which providers can use AI to maximize time spent with patients. Smart-scheduling, auto-coding and charge capture are all technology-driven solutions to the problems created by a complex health care system with an ever-increasing administrative load. If HHS wishes to accelerate the use of AI, it could encourage CMS to expand coverage for ambient AI and other technologies that reduce provider pain points.

Clinical Decision Support Systems and Diagnostic Tools: If appropriately regulated, clinical decision support systems and diagnostic tools have the most potential in augmenting the clinical decision-making process and improving patient outcomes. A clinical decision support is a digital tool that provides timely and person-specific information to enhance patient outcomes and quality of care. It can assist the therapist in selecting the most appropriate treatment by [identifying potential diagnoses, suggesting appropriate tests, and recommending evidence-based treatments](#). It can serve as a clinical assistant by helping the therapist synthesize complex information and consider diagnostic possibilities. The goal is to enhance clinical reasoning while allowing the PT to maintain human oversight of final decisions regarding treatment. While these tools demonstrate the highest potential, they must be regulated appropriately. As such, APTA recommends that HHS ASTP/ONC establish certification criteria for AI-enabled clinical decision support tools that require disclosure of training data characteristics, model performance on diverse populations, and explanation of decision logic.

Patient Engagement Tools: Patient engagement tools, such as patient portals, digital platforms, and mobile apps, can serve as asynchronous touch points between therapy visits. While remote therapeutic monitoring codes already provide reimbursement for patient monitoring using wearable technology, mobile apps and other tools can improve the patient experience by streamlining communication and personalizing healthcare. Some patients may even better adhere to their treatment plans. Many of these tools are already widely available and used in healthcare settings.

The following tools should be used cautiously or not at all:

Computer Vision Technology and Independent Triage Tools: There is immense value in computer vision technology that can document the patient's physical capabilities during and after the therapy session. If computer vision is used during the session, the therapist can give their full attention to the patient and can review the patient's performance after the fact as needed. While technology cannot fully replicate the observational abilities and critical thinking of the therapist, it can enhance the capacity of the therapist during the session and maximize the time the PT spends with the patient. In these instances, the technology could also generate notes and a post-visit summary, which the therapist can review, edit appropriately, and sign-off on. This technology has immense potential to reduce burden and maximize both the patient and provider experience.

These tools can also be deployed outside of the clinic, in the patient's home. Patients can use computer vision and wearable devices to better understand their performance as they complete home exercise programs. Certain tools contain feedback applications that report patterns to the therapist who can appropriately decide the next course of action. Like wearable devices that analyze movements, feedback applications can flag when a patient's joint moves out of alignment. For example, if AI technology were to provide real-time joint angle feedback, the patient could correct an improper movement in real-time, rather than wait until their next in-person visit. However, if there is no mechanism triggering providers to sign-off, this may not be a safe and effective tool. As such, APTA recommends caution when using triage tools such as home exercise feedback applications and progress dashboards that do not appropriately triage patient performance or consult a therapist before making changes to the treatment plan that should be overseen by a licensed professional.

Fully Autonomous AI: APTA does not support the use of fully autonomous diagnosis bots or the provision of unsupervised treatment plans. In clinical settings, human oversight ensures data accuracy and patient safety. If there is no clinician sign-off or approval processes in place, AI technology is acting, for all intents and purposes, as a physical therapist. And APTA believes that AI will be most effective when it is used to support clinicians rather than operate independently. Any clinical tools that use agentive AI, which has minimal human oversight and independently handles complex, multi-step workflows, should be rigorously reviewed and only cautiously deployed in clinical care, if at all.

AI in Prior Authorization and Utilization Management Processes: While the use of AI in utilization management could potentially lead to quicker access to care and reduce provider and plan burden, it poses numerous risks if utilized inappropriately. As such, [APTA opposes any use of AI by payers to deny or restrict access to, or reduce payment for, physical therapist services or other medically necessary services.](#)

While the [Final Rule on Interoperability and PA](#) aims to speed up PA processes and create more transparency in PA criteria and denials, it does not regulate how such decisions are made, particularly with or without AI. This level of transparency is exceptionally important as insurers make decisions that determine if patients receive care, the type or intensity of the care, and the location of care, and when information on how those decisions are made is not publicly available, patients, providers, and researchers are unable to determine whether the use of AI produces better or worse outcomes.

This is particularly concerning given the dramatic increase in the use of AI tools by health insurers. A [2024 survey](#) by the National Association of Insurance Commissioners of 93 large health insurers found that 84 percent were using AI for operational purposes. More specifically, thirty-seven percent of insurers report using AI for PA and 56 percent for utilization management. Yet as stated in previous sections, AI tools are only as accurate as the data and algorithm inputs going into them. If used appropriately and effectively, AI could automate PA and claims approval, reduce denials when there is incomplete information, and assist with the complex appeals process. But if any AI tools are to be used

in PA, they need accurate clinical criteria and appropriate levels of human review, otherwise, AI may only serve to exacerbate existing flaws.

Some insurers have found success using AI. Cohere Health has [leveraged clinically trained AI to automate PA requests while still maintaining 94% provider satisfaction](#). Cohere allows clinicians to use Dragon Copilot during patient visits to instantly submit care requests and receive real-time feedback from health plans. It should be noted that Cohere, following CMS' guidelines, does not use AI alone for denials; when a denial is processed, a human is always in the loop. But unfortunately, Cohere has begun to deny services that the therapist provides on the same day as the initial evaluation even though PA is not required by the patient's plan. Small glitches in the system such as these can be magnified by the use of AI in PA.

While Cohere has seen high rates of satisfaction, many plans have used AI with little to no human review resulting in systemic batch denials. The most well-known case occurred within Medicare Advantage where three health plans, UnitedHealthcare, CVS, and Humana, saw [denial rates jump](#) after implementing AI in their PA processes. Notably, UnitedHealthcare's system, nH predict, reported a 90% error rate for post-acute care stays, leading to [a lawsuit against the company](#). This is not surprising given that in some cases, AI tools report [16 times higher rates of care denial](#).

This practice concerns both physicians—[61% of whom said they fear that payers' use of unregulated AI is increasing prior authorization denials](#)—and non-physician providers alike who are expressly concerned with the degree to which PA already delays care or denies access to medically necessary services. And even if PA processes are appropriately and effectively streamlined using AI, they are still a severe impediment to patient's receiving timely care. In November 2025, APTA released [The Impact of Administrative Burden on Physical Therapist Services](#), our third and most recent survey of administration burden within the profession. The report found that:

- Wait times for prior authorization have escalated steadily over the last seven years, with 30% of survey respondents waiting one to two weeks for prior authorization approval.
- Prior authorization requirements negatively impacted patients' clinical outcomes with 83% of respondents agreeing or strongly agreeing that prior authorization has caused their patients to abandon treatment.
- 75% of respondents said they had to hire administrative staff to keep up with the administrative demands.
- 57% agreed or strongly agreed that administrative burden has led their practice to discontinue participation with a payer or network.

This data is alarming and serves as a cautionary tale. CMS has already [clarified](#) that while MA plans can use AI to make coverage determinations, “an algorithm that determines coverage based on a larger data set instead of the individual patient's medical history, the physician's recommendations, or clinical notes” would not be compliant with federal law protecting Medicaid and Medicare beneficiaries, yet APTA is deeply concerned that AI will only exacerbate existing challenges in PA. APTA also affirms its

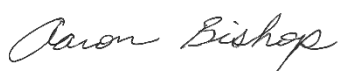
stance that AI should never be the sole reason for a denial and the use of AI based solely on utilization statistics should never be used to deny care. To prevent this from happening, APTA encourages HHS to provide the following oversight to Medicare, Medicaid, and Affordable Care Act Marketplace plans that utilize AI in PA:

- **Pre-Validation Scrutiny:** In creating algorithms for AI in PA, insurers should apply scrutiny to all processes, but APTA is particularly concerned about those AI tools that pose the greatest risk of harm by erroneously suggesting that treatment requests are not approvable such as denying care at a facility altogether. Pre-validation review should ensure that any denials are automatically routed to a human for clinician review in a timely manner.
- **Continuous Monitoring and Oversight:** HHS should set standards for insurers who deploy AI in PA. Specifically, we recommend HHS require insurers to scrutinize requests that are persistently denied or entail prolonged delays in approval to identify commonalities and fix algorithmic determinations when necessary.
- **Human Review:** Medicare Advantage plans and many state-regulated plans are required by law to ensure that no request is denied without having been reviewed by a medical professional. While many plans have [consistently affirmed](#) that they are compliant with this requirement, APTA encourages further transparency in the approval process. Insurers should be honest about when a human does or does not take part in the approval process and should provide some sort of verification to this effect.
- **Opacity in Algorithmic Determinations:** Insurers have pledged to provide clearer explanations of prior authorization denials, and HHS should hold them accountable. As mentioned above, there are no regulations governing how decisions are made which can lead to frustration and arduous appeals for the provider and patient. HHS should consider requiring more transparency when AI is used to adjudicate requests.

As the Centers for Medicare and Medicaid Innovation launches the [Wasteful and Inappropriate Service Reduction Model](#), we encourage HHS to closely monitor the use of AI and machine learning to ensure that it is appropriately paired with clinical review and that care is not unnecessarily delayed through the model. We are eager to learn about the results that WISeR produces and ask that the HHS proactively engage with CMMI to learn about how WISeR addresses many of the concerns discussed above.

APTA appreciates the opportunity to provide feedback on HHS' Request for Information regarding accelerating the adoption and use of artificial intelligence as part of clinical care. Should you have any questions regarding our comments, please contact Rachel Miller, MPH, senior specialist in health policy and payment at rachelmiller@apta.org. Thank you for your time and consideration.

Sincerely,

A handwritten signature in black ink that reads "Aaron Bishop".

Aaron Bishop
APTA Chief, Public Affairs